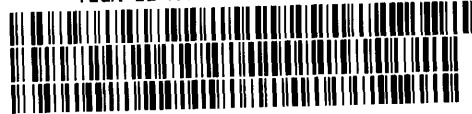


Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
ILP/PA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary		
Case is (circle):	QUALIFIED / DISQUALIFIED	
Reviewer: Initials	Date Reviewed: 9/27/12	

UID: 8EC3C2D5-F169-4C1D-B38F-536FCF1FDD67
TCGA-EL-A4K6-01A-PR Redacted



DOB:
Physician:

Sex: M

Accession:

ICD-0-3

Collected:

Received:

Case type: Surgical Case

DIAGNOSIS

(A) LYMPH NODE, LEFT PARATRACHEAL, EXCISION:

Three lymph nodes, no tumor present (0/3)

Path: Carcinoma, papillary and follicular 8340/3

Site: thyroid, NOS C73.9

10/4/12

CRCF: carcinoma, papillary, thyroid 8260/3

(B) THYROID, TOTAL THYROIDECTOMY, PARATRACHEAL AND SUPERIOR MEDIASTINAL CONTENTS, AND RIGHT NECK DISSECTION:

PAPILLARY THYROID CARCINOMA, WITH A DOMINANT FOLLICULAR COMPONENT

Location: Right lobe

Multi-focal: No

Size = 6.2 cm

Extrathyroidal extension: Present

Lymph nodes:

METASTATIC PAPILLARY THYROID CARCINOMA IN 3 OF 39 LYMPH NODES

Location: Right and central, neck

Extracapsular extension: Present

FOCUS OF PAPILLARY THYROID CARCINOMA, FOLLICULAR VARIANT

Location: Left lobe

Size = 0.4 cm

Follicular adenoma

US 70 per TSS.

GROSS DESCRIPTION

(A) LEFT PARATRACHEAL LYMPH NODE – Specimen consists of three possible lymph nodes, 0.2 cm in greatest dimension. Specimen is entirely submitted in A.

(B) RIGHT NECK DISSECTION, TOTAL THYROID, PARATRACHEAL, SUPERIOR MEDIASTINAL CONTENTS - A total thyroidectomy (17.0 x 10.5 x 4.2 cm) with attached central and right modified neck dissections (11.0 x 9.0 x 2.0 cm). Left thyroid lobe, 5.0 x 2.7 x 1.0 cm, isthmus, 2.0 x 2.0 x 0.4 cm, and right lobe, 8.0 x 6.0 x 4.5 cm.

The right lobe consists of a dominant tumor nodule (6.2 x 5.0 x 4.1 cm) with variegated red-tan and yellow soft parenchyma with intra-fibrous septa and a thick capsule. There is gross extension of the tumor through the capsule with possible matted lymph nodes at approximately the mid lobe.

The left lobe is remarkable for a firm, partially calcified red-tan nodule (1.1 x 0.9 x 0.7 cm) at the superior aspect with an adjacent soft pink-tan nodule (0.7 cm in maximum diameter). A separate well circumscribed red-tan, partially cystic nodule (0.6 cm in maximum diameter) is located in the inferior portion of the right lobe.

The central neck fibroadipose tissue is remarkable for seven possible lymph nodes, some of which appear to be multiple matted lymph nodes (up to 1.4 cm in maximum dimension). The fibroadipose tissue of the right neck has multiple lymph nodes (up to 1.5 cm in maximum dimension), some of which reveal variegated red-tan and pink parenchyma that may represent tumor involvement.

DOB:
Physician:

Sex: M

Collected:

Pathologist:

Accession:

Received:

Case type: Surgical Case

Representative sections are submitted. Representative sections of tumor and possibly involved lymph node from the central neck are submitted to the tissue bank.

SECTION CODE: B1-B14, right neck lymph nodes (B1-B9, single lymph node, bisected, in each; B10-B12, six possible lymph nodes, in each; B13, seven possible lymph nodes; B14, eight possible lymph nodes); B15-B20, central neck lymph nodes (B15, six possible lymph nodes, B16, one possible lymph node, bisected, B17, multiple matted lymph nodes, serially sectioned, B18-B20, possible matted lymph nodes); B21-B24, right neck lymph nodes (B21, four possible lymph nodes; B22, five possible lymph nodes; B23, six possible lymph nodes; B24, one possible lymph node); B25-B29, left thyroid lobe sampled from superior to inferior (B25-B26, left lobe superior calcified nodule and soft tan nodule; B28, second inferior nodule); B30, isthmus, representative; B31-B40, right lobe submitted from superior to inferior (B35-B39 including possible matted lymph nodes; B36-B37, contiguous sections; B38-B39, contiguous sections; B40, inferior aspect of lobe); total 40.

CLINICAL HISTORY

None given.

SNOMED CODES

T-B6000, M-80503, T-C4200, M-80506,

"Some tests reported here may have been developed and performance characteristics determined by
These tests have not been specifically cleared or approved by the U.S. Food and Drug Administration."

Entire report and diagnosis completed by:

Page 2 of 2	
Surgical Pathology Report	
File under: Pathology	